

KANGAROO MOTHER CARE

Kangaroo Mother Care India Network

What is Kangaroo Mother Care (KMC)?

Skin to skin contact between baby and mother's chest



Provides warmth, prevents aspiration, stimulates breathing

How to provide KMC?

Mother should be in sitting or semi reclined position



Provide KMC for as long as possible

Which babies need KMC?

All stable LBW babies are eligible for KMC



All LBW babies benefit from KMC

Where can KMC be provided?



Nursery or postnatal ward

How long should KMC be practiced?



Should be continued at home

Who else can provide KMC?



Any family member can provide KMC

What are the components of KMC?

Skin-skin contact of the infant on mother's chest



Exclusive breast feeding



Pre-requisites of KMC

Support to mother & post-discharge follow-up



Benefits

- Provides warmth to the baby
- Promotes exclusive breast feeding
- Improves weight gain & growth
- Reduces hospital stay
- Reduces infection
- Promotes baby-mother bonding



What are you waiting for?

KMC is a simple, low-cost and highly effective intervention which benefits low birth weight babies

- The babies and their mother love KMC
- You too can promote KMC in your unit ... start today ...
- Ensure KMC for all stable LBW babies

For more information visit www.kmcindia.org



Prepared by : CoE, State Institute of Health & Family Welfare, Odisha



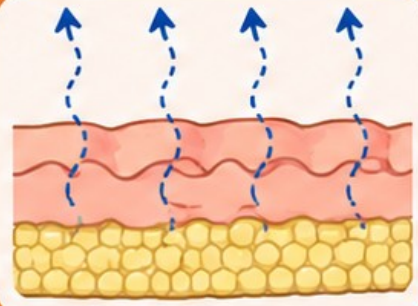
Thermal Control (KMC)

Topic: 3

Why are newborns prone to develop hypothermia?



Larger surface area



Decreased thermal insulation due to lack of subcutaneous fat



Reduced amount of brown fat

METHODS OF HEAT LOSS IN BABY

Newborns lose heat faster due to larger surface area, thin skin and limited body fat.

RADIATION

Heat loss from the body to surrounding objects without direct contact.

CONVECTION

Heat loss from the body to the surrounding air in movement.

EVAPORATION

Heat loss through skin when moisture evaporates.

CONDUCTION

Heat loss through direct contact with a cooler surface.



PREVENT HEAT LOSS



Keep the baby warm and covered



Maintain warm environment



Keep skin dry and clean



Handle gently and minimize exposure

ASSESSMENT OF TEMPERATURE BY TOUCH

Feel by touch Trunk	Feel by touch Extremities	Interpretation
Warm	Warm	Normal
Warm	Cold	Cold stress
Cold	Cold	Hypothermia

NEONATAL HYPOTHERMIA: ASSESSMENT & MANAGEMENT

Recognize early. Act promptly. Prevent complications

 CATEGORY	 TEMP. RANGE (Axillary)	 FEEL BY TOUCH	 CLINICAL FEATURES	 ACTION
 NORMAL	36.5 to 37.4°C	• Warm trunk • Warm extremities	 Normal baby	• Cover adequately with pre-warmed cloth • Keep the baby next to mother • Encourage breast feeding
 MILD HYPOTHERMIA (COLD STRESS)	36 to 36.4°C	• Warm trunk • Cold extremities	 Extremities bluish and cold Poor weight gain if chronic cold stress	• Skin-to-skin contact • Cover adequately • Ensure room is warm • Provide warmth • Encourage breast feeding
 MODERATE HYPOTHERMIA	32 to 35.9°C	• Cold trunk • Cold extremities	 • Poor sucking • Lethargy • Weak cry • Fast breathing	• Cover mother and baby together using pre-warmed clothes • Cover adequately • Provide warmth • Reassess every 15 minutes; if temperature doesn't improve, provide additional heat • Encourage breast feeding
 SEVERE HYPOTHERMIA	Less than 32°C	• Cold trunk & cold extremities	 • Lethargic • Poor perfusion • Fast or slow breathing • Slow heart rate • Hardening of skin with redness and edema • Bleeding • Low blood sugar	• Rapid re-warming till baby is 34°C and then slow re-warming • Oxygen • IV fluids – dextrose (warm) • Inj. vitamin K • Reassess every 15 minutes; if temperature doesn't improve, provide additional heat

METHODS OF HEAT LOSS IN NEWBORNS



RADIATION
Heat loss to colder surroundings without direct contact.



CONVECTION
Heat loss to moving air or currents around the baby.



CONDUCTION
Heat loss through direct contact with cold surfaces.



EVAPORATION
Heat loss due to evaporation of moisture from the skin.



PREVENTING HEAT LOSS: KEY SITUATIONS & PRACTICAL STEPS



COMMON SITUATIONS WHERE COLD STRESS CAN OCCUR



At birth



After giving bath



During changing of nappy/clothes



Malfunctioning heat source or
removing the baby from heat source



While transporting a sick baby



STEPS TO PREVENT HEAT LOSS IN LABOR ROOM



Warm delivery
room (25°C)



Newborn care corner
temperature to be
maintained at 30°C



Drying immediately.
Dry with one towel.
Remove the wet towel
and cover with another
pre-warmed towel



Skin-to-skin contact
between mother
and baby



STEPS TO PREVENT HEAT LOSS IN POSTNATAL WARD



Breastfeeding



Appropriate clothing,
cover head and
extremities



Keep mother and
baby together



Keep room warm



Postpone bathing
and weighing



Maintaining warmth protects newborns from cold stress and supports healthy survival and growth.

WARM CHAIN



- The “warm chain” is a set of interconnected procedures carried out at birth and later which will minimize the likelihood of hypothermia in all newborns.



- Baby must be kept warm at the place of birth (home or hospital) and during transportation from home to hospital or within the hospital.



- Satisfactory control of baby’s temperature demands both prevention of heat loss and providing extra heat using an appropriate source.

WARM ENVIRONMENT

Ensure a warm room and minimal exposure.



WARM DURING TRANSPORT

Keep the baby warm during transportation.



KEEP BABY WARM

APPROPRIATE WARMTH

Use external heat source when needed.



IMMEDIATE DRYING

Remove wet cloths and dry the baby immediately.



SKIN-TO-SKIN CONTACT

Place baby on mother’s bare chest and cover both.



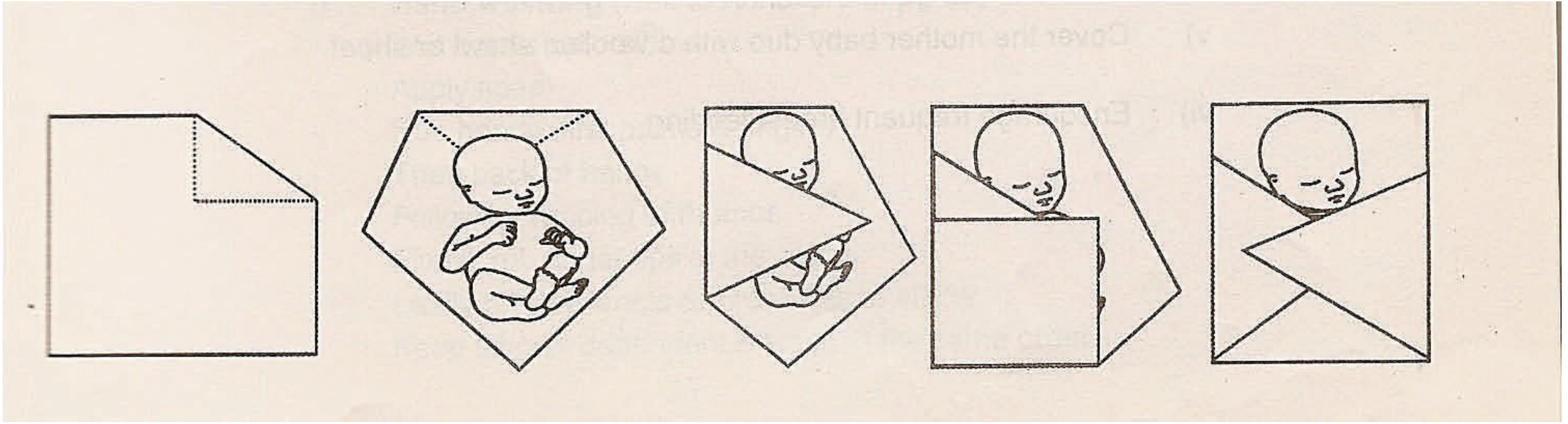
KEEP COVERED

Cover the head, use a cap, and wrap the baby properly.



A WARM BABY TODAY, A HEALTHY BABY TOMORROW.

Wrapping and covering a baby



KANGAROO MOTHER CARE

Kangaroo Mother Care (KMC) is a special way of caring for the low birth weight (LBW) babies.



COMPONENTS OF KANGAROO MOTHER CARE (KMC)



**Skin-to-skin
contact**



**Exclusive
breastfeeding**

PRE-REQUISITES OF KANGAROO MOTHER CARE (KMC)



**Support to the
mother in hospital
and at home**



**Post-discharge
follow up**

Benefit of KMC

(Kangaroo Mother Care – Care Close, Love Deep)



ELIGIBILITY CRITERIA



All **stable LBW** babies are **eligible** for KMC.

However, very sick babies needing special care should be cared under radiant warmer initially.



KMC can be initiated in a baby who is otherwise **stable** but may still be on **intravenous fluids, tube feeding** and/or **oxygen**.



KMC saves lives, promotes growth, and strengthens the bond between mother and baby.



THE KMC PROCEDURE



KANGAROO POSITIONING



Baby should be placed between the mother's breasts in an **upright position**.



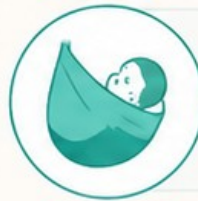
Head should be **turned to one side** and in a slightly extended position.



Hips should be **flexed and abducted** in a "frog" position.

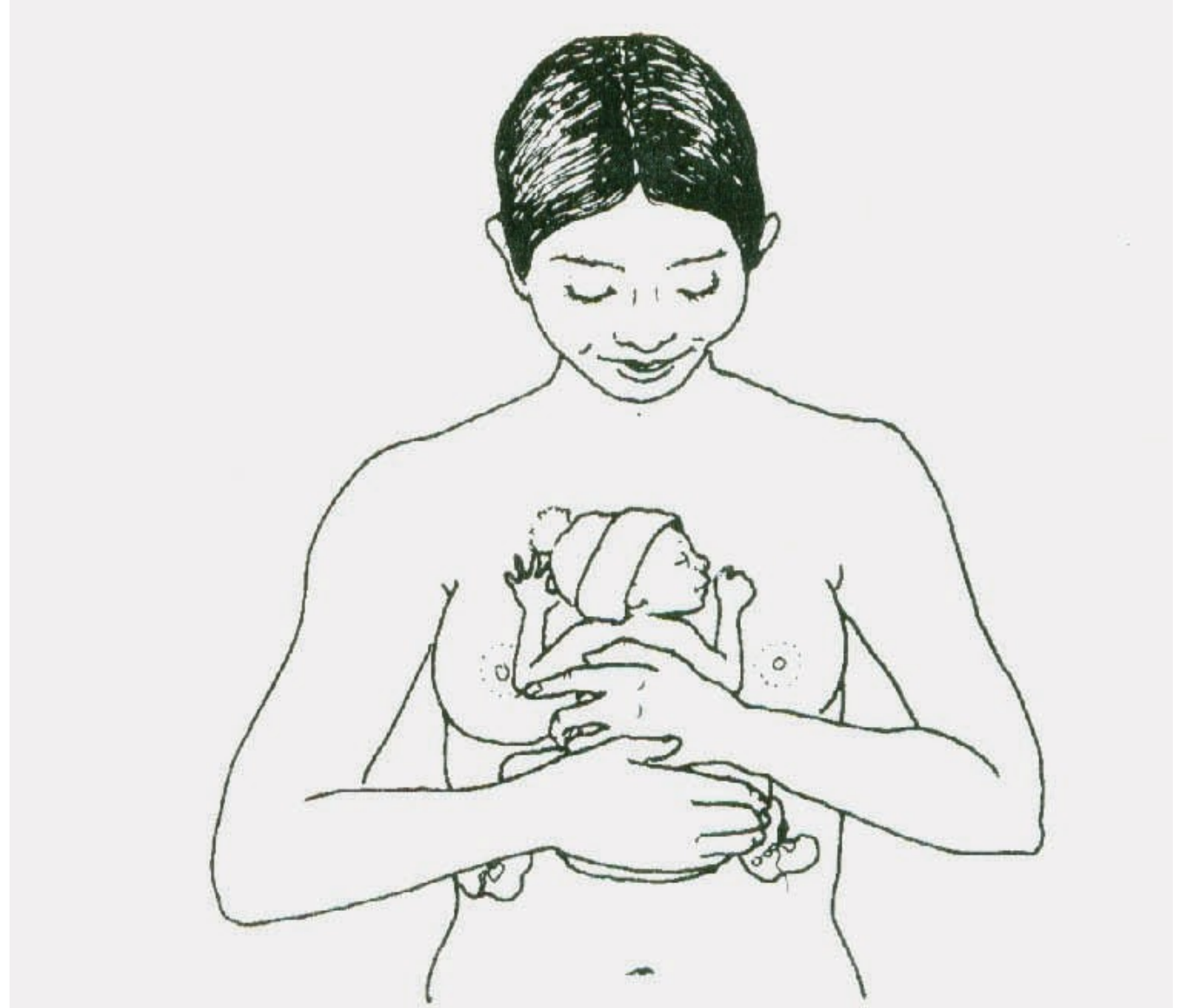


Baby's abdomen should be at the level of the mother's **epigastrium**.



Support the baby's bottom with a **sling/binder**.

Kangaro o position ing



THE KANGAROO METHOD



Place baby in this position

Then cover with clothes

Monitoring

Ensure that baby's neck is not too flexed or too extended, breathing is normal, and feet and hands are warm during KMC



Duration of KMC



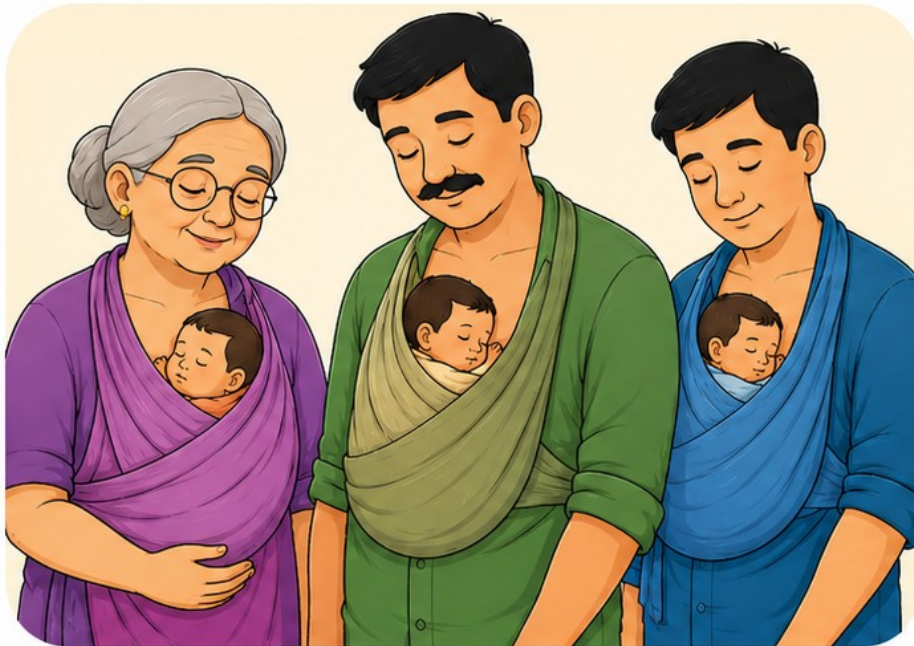
Sessions that last less than one hour should be avoided



The length of skin-to-skin contacts should be gradually increased up to 24 hours a day, interrupted only for changing diapers.

WHEN MOTHER IS NOT AVAILABLE

OTHER FAMILY MEMBERS CAN PROVIDE KMC



WHO CAN PROVIDE KMC?



When the mother is not available, other family members such as

GRANDMOTHER, FATHER OR OTHER RELATIVE

can provide Kangaroo Mother Care (KMC).



SKIN-TO-SKIN CONTACT

Ensure baby is in direct contact with the bare chest and covered properly.

BENEFITS OF KMC BY FAMILY MEMBERS



Keeps baby warm



Improves baby's health and survival



Promotes better weight gain



Supports better breathing



Strengthens bond and family involvement

FEEDING OF NORMAL AND LOW BIRTH WEIGHT BABIES



Enteral feeding of normal birth weight babies and low birth weight babies

Breastfeeding counselling and support

Managing common problems encountered during breastfeeding

Feeding by Paladai

